



University
of Glasgow

From populations to individuals and back again

Andrew V. Biankin

Regius Professor of Surgery

Executive Director, International Cancer Genome Consortium

Chair, Precision-Panc Therapeutic Development Platform

University of Glasgow



Realising the Promise of Precision Oncology

On our Planet.



Every year...

**9.6 million people with cancer
run out of options...**

...and die.

Every year...

**Less than 1 patient out of 100 will access
new treatments or clinical trials.**

Every year...

Almost all clinical trials fail to reach recruitment targets, and most never complete.

Every year.

**Most potential treatments
are never tested**

sit on a shelf

Lost.

And nothing changes.

Year after year.

Trillions of tears.

**Tamoxifen, the treatment that has helped
dying cancer patients more than all other
drug treatments combined**

Almost by accident.

Was rescued from a dusty shelf

**We have all the drugs we need to double
survival for cancer**

**We test only a small fraction of them,
in a fraction of cancer types**

So what is the problem...

Are we the problem.

A combination of total luck, a great oncologist and determination to live has enabled me to access my current trial and get an extra four years of life I didn't expect to have.

As a result I've seen my eldest go to uni, the other three all grow up.

All because I chanced upon a drug that precisely targeted my cancer.

*Lesley Stephen
Cancer Patient
Edinburgh, 2019*

In just 1 hour of our meeting today...

**20 people in the United Kingdom
will die of cancer.**

1000 people on our planet.

We need to do something...

What future can we build?



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**Parachutes have never been proven to work
in a randomised controlled trial**

**“Can you believe that not that long ago
we would bleed people and do other
barbaric things to treat them?
How crazy is that.”**

Member of the public 2025

**“Can you believe that not that long ago
we gave toxic chemicals to people to
treat diseases without knowing if they
would work or not.”**

Member of the public 2075

Precision Oncology – a natural evolution of healthcare

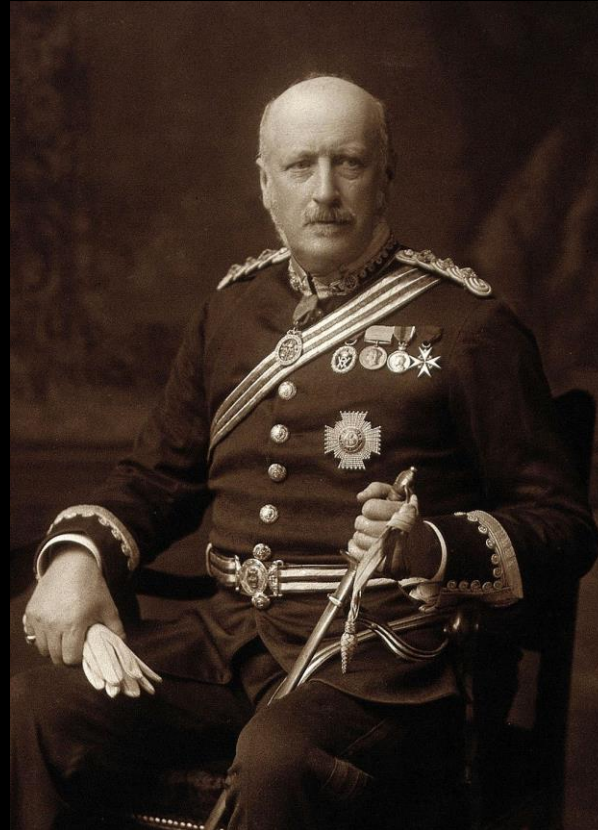
First example of Targeted Therapy: George Beatson
– Oophorectomy for Breast Cancer. *The Lancet* July 18, 1896

ON THE TREATMENT OF INOPERABLE CASES OF CARCINOMA OF THE MAMMA: SUGGESTIONS FOR A NEW METHOD OF TREATMENT, WITH ILLUSTRATIVE CASES.¹

BY GEORGE THOMAS BEATSON, M.D. EDIN.,
SURGEON TO THE GLASGOW CANCER HOSPITAL; ASSISTANT SURGEON
GLASGOW WESTERN INFIRMARY; AND EXAMINER IN SURGERY
TO THE UNIVERSITY OF EDINBURGH.

(Concluded from page 107).

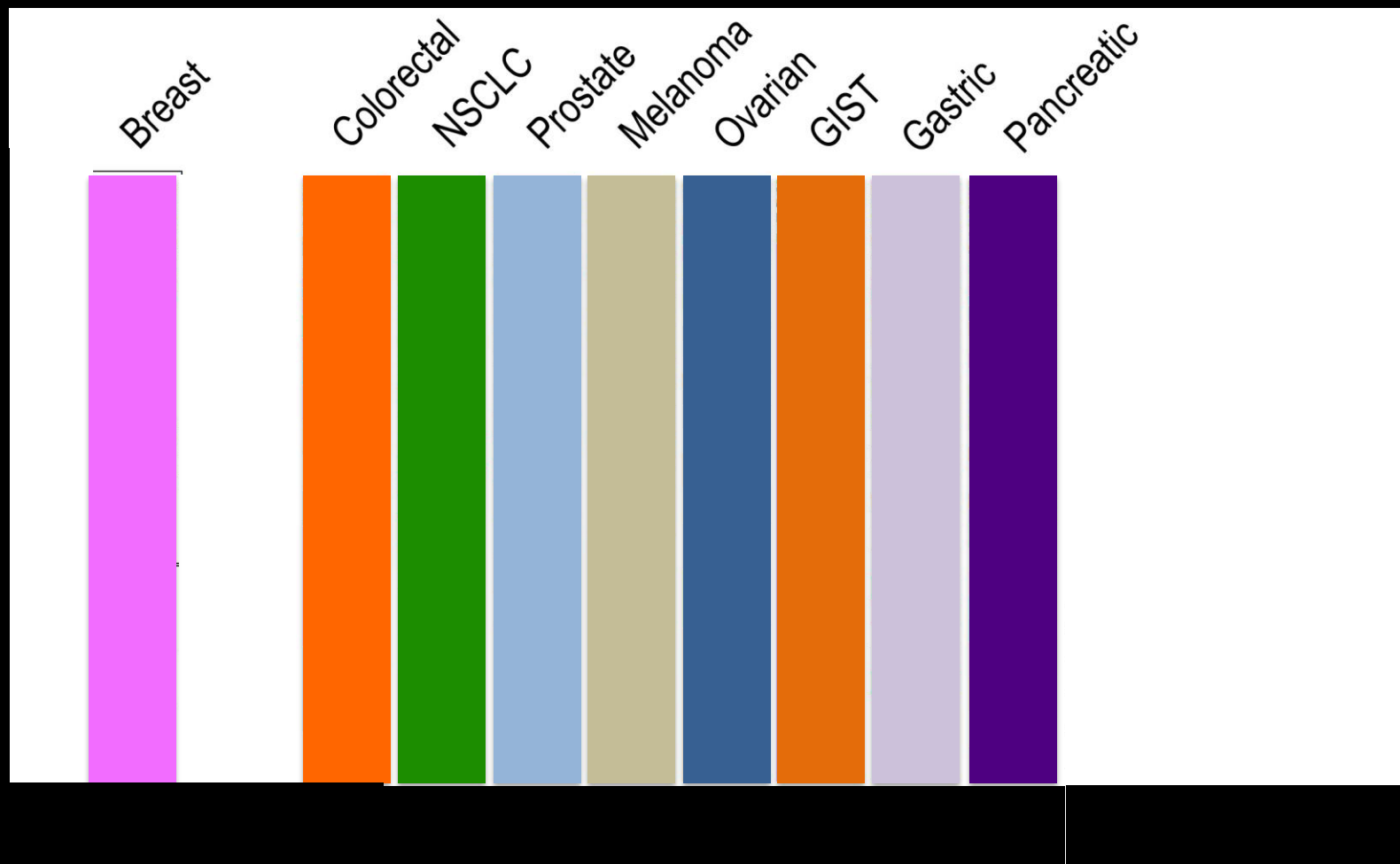
THE next case that I wish to bring under notice is that of a married woman aged forty years, with no family, who was admitted to the Glasgow Cancer Hospital on Sept. 2nd, 1895, suffering from a large tumour of the right mamma. It had



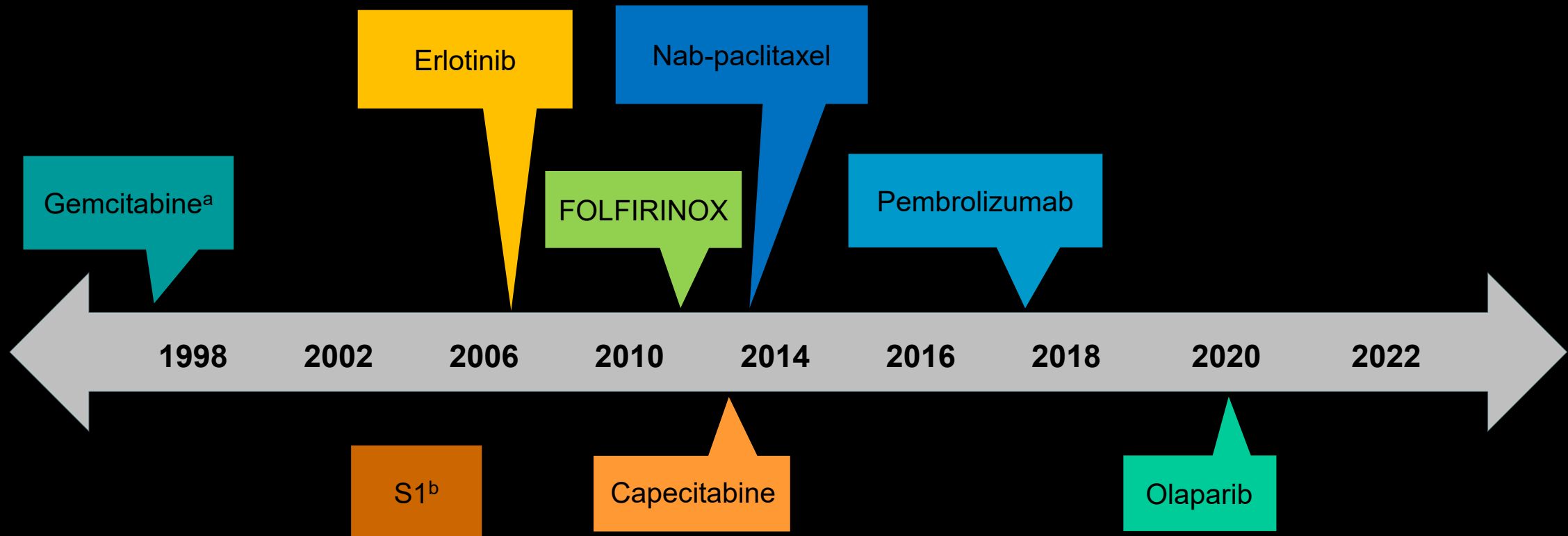
Most Successful
Targeted Therapy:
Tamoxifen (Developed
in the 1970's)



Cancer is complex



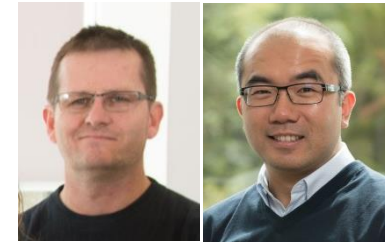
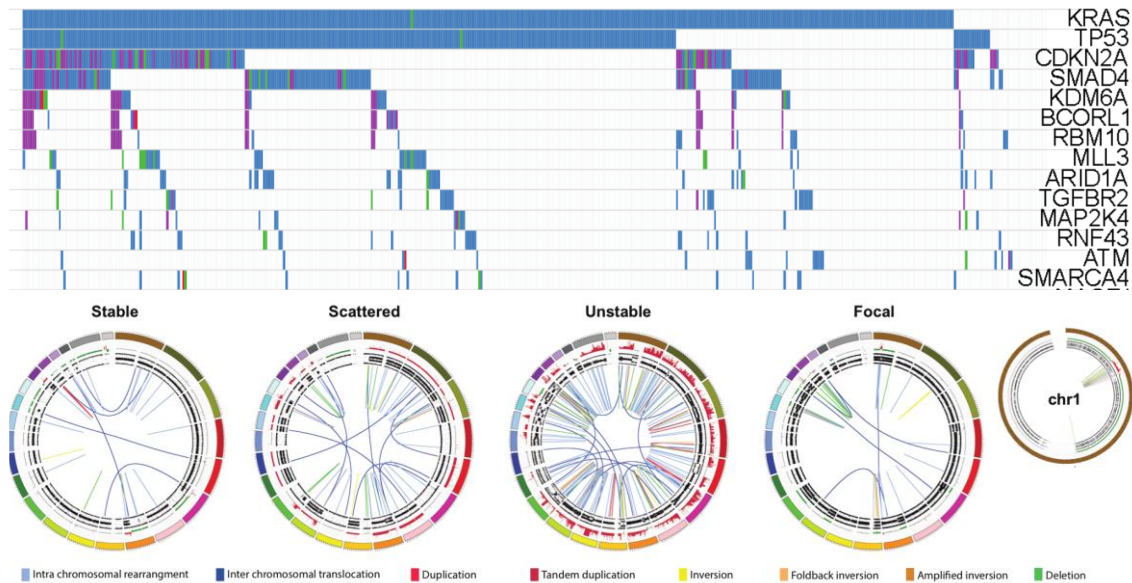
Treating pancreatic cancer: Increasing availability of therapies with incremental benefit



^a Subsequently, gemcitabine-based combinations (eg, gemcitabine/capecitabine) also validated as treatment options in metastatic disease.^{1,2} ^b Approved only in Japan.

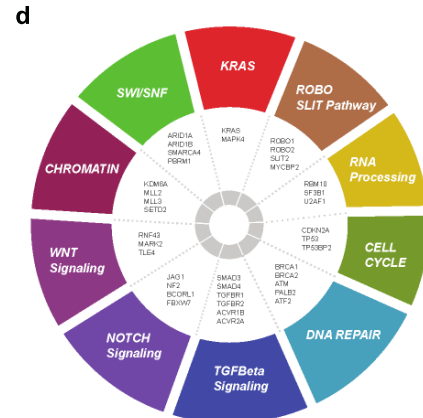
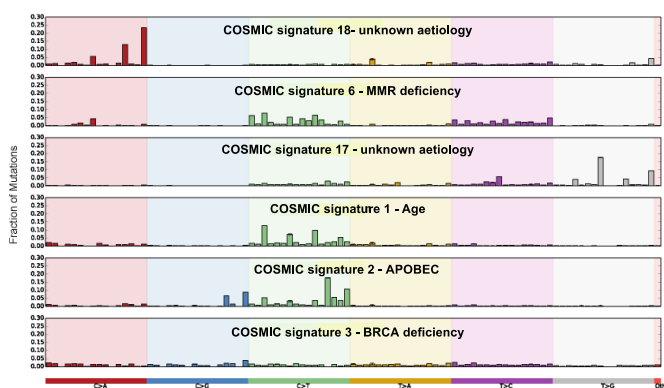
1. Herrmann R et al. *J Clin Oncol*. 2007;25:2212-2217. 2. Cunningham D et al. *J Clin Oncol*. 2009;27:5513-5518.

Key Molecular Mechanisms and Potential Novel Vulnerabilities (n = 457 multiplatform -omic analysis)



Peter Bailey
University of Glasgow

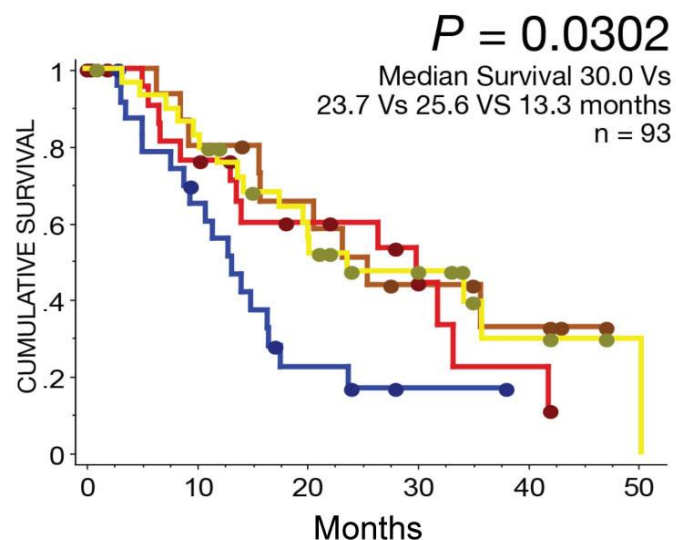
David Chang
University of Glasgow



Hudson et al. Nature 2010
Biankin et al. Nature 2012
Perez-Mancera et al. Nature 2012
Mann et al. PNAS 2012
Alexandrov et al. Nature 2013
Nones et al. IJC 2013
Weissmueller et al. Cell 2014
Waddell et al. Nature 2015
Biankin et al. Nature 2015
Bailey et al. Nature 2016
Scarpa et al. Nature 2017
Feigin et al. Nature Genetics 2017
Balachandran et al. Nature 2017
TCGA Cancer Cell 2017

Transcriptomic Subtypes of PDAC

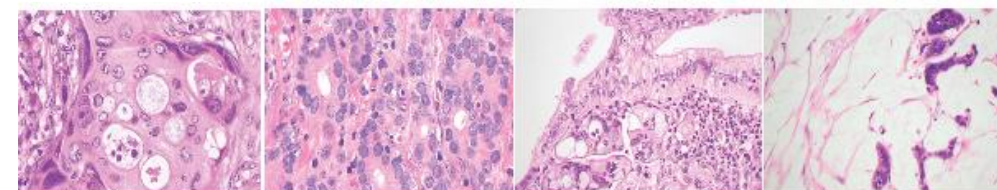
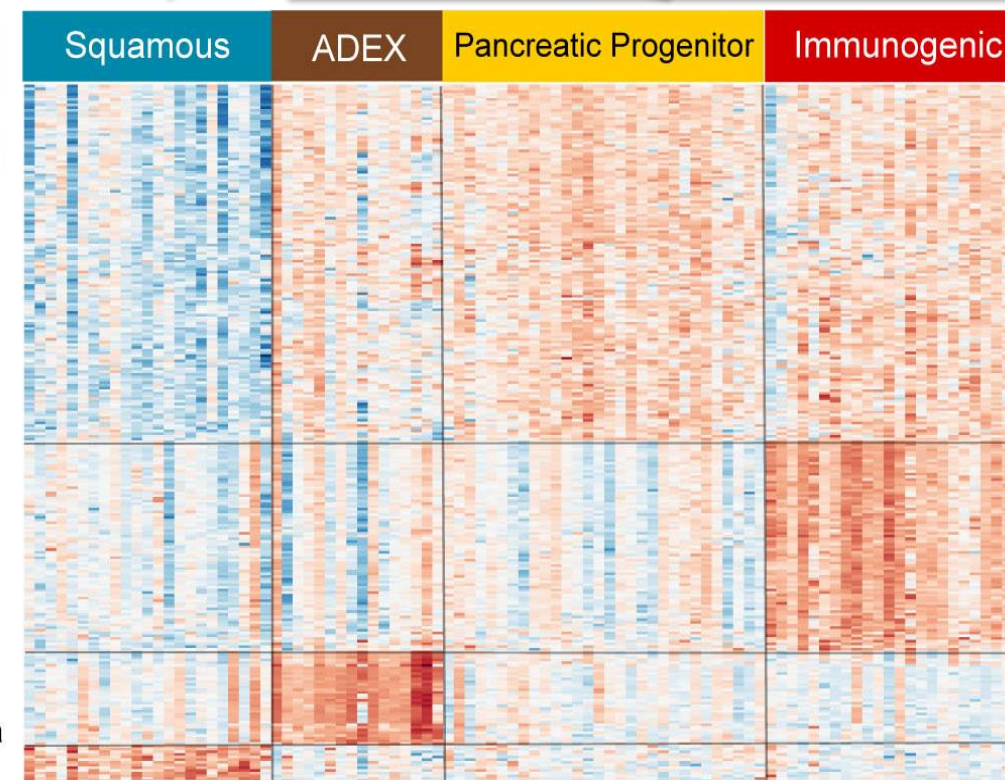
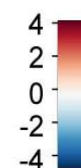
Bailey *et al. Nature* 2016



N = 96; Discovery RNAseq

N = 261; Validation expression array

Scattered
Focal
Stable
Unstable
Basal-like
Classical
Activated stroma
Normal stroma

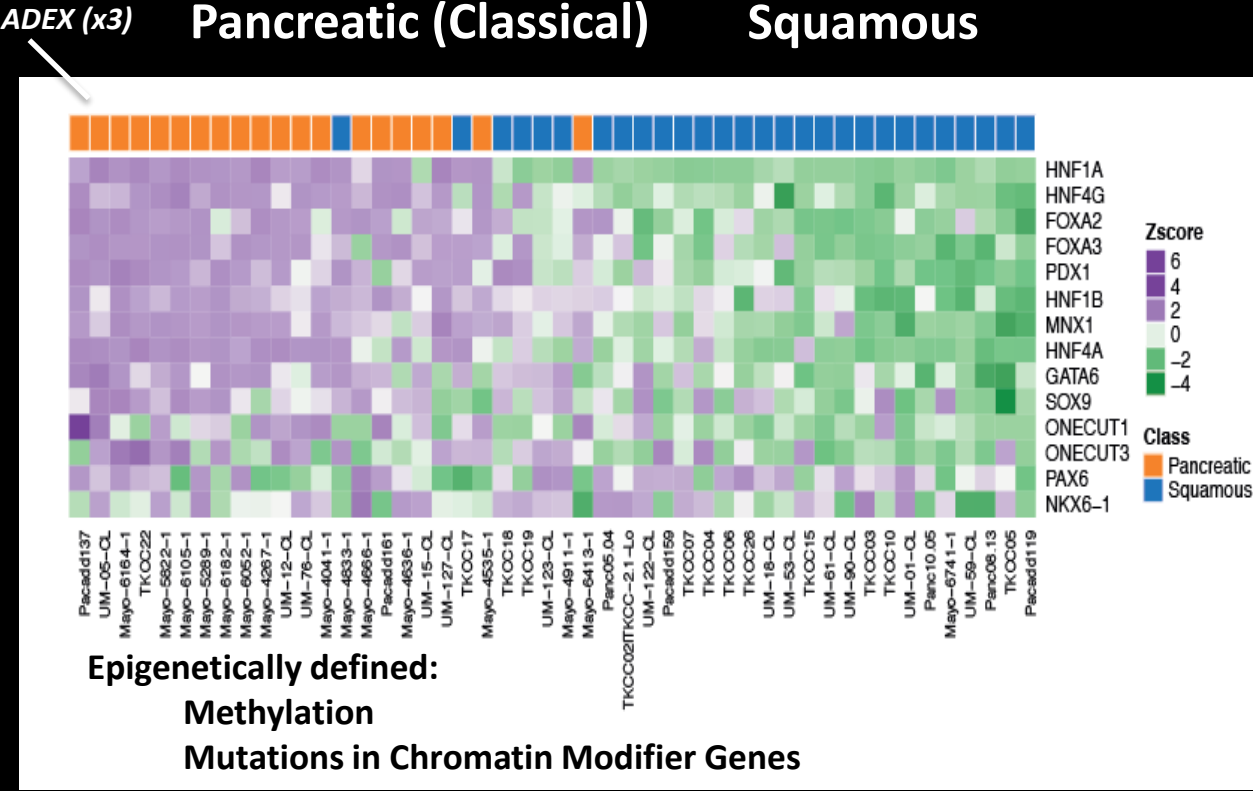


Adenosquamous
Carcinoma

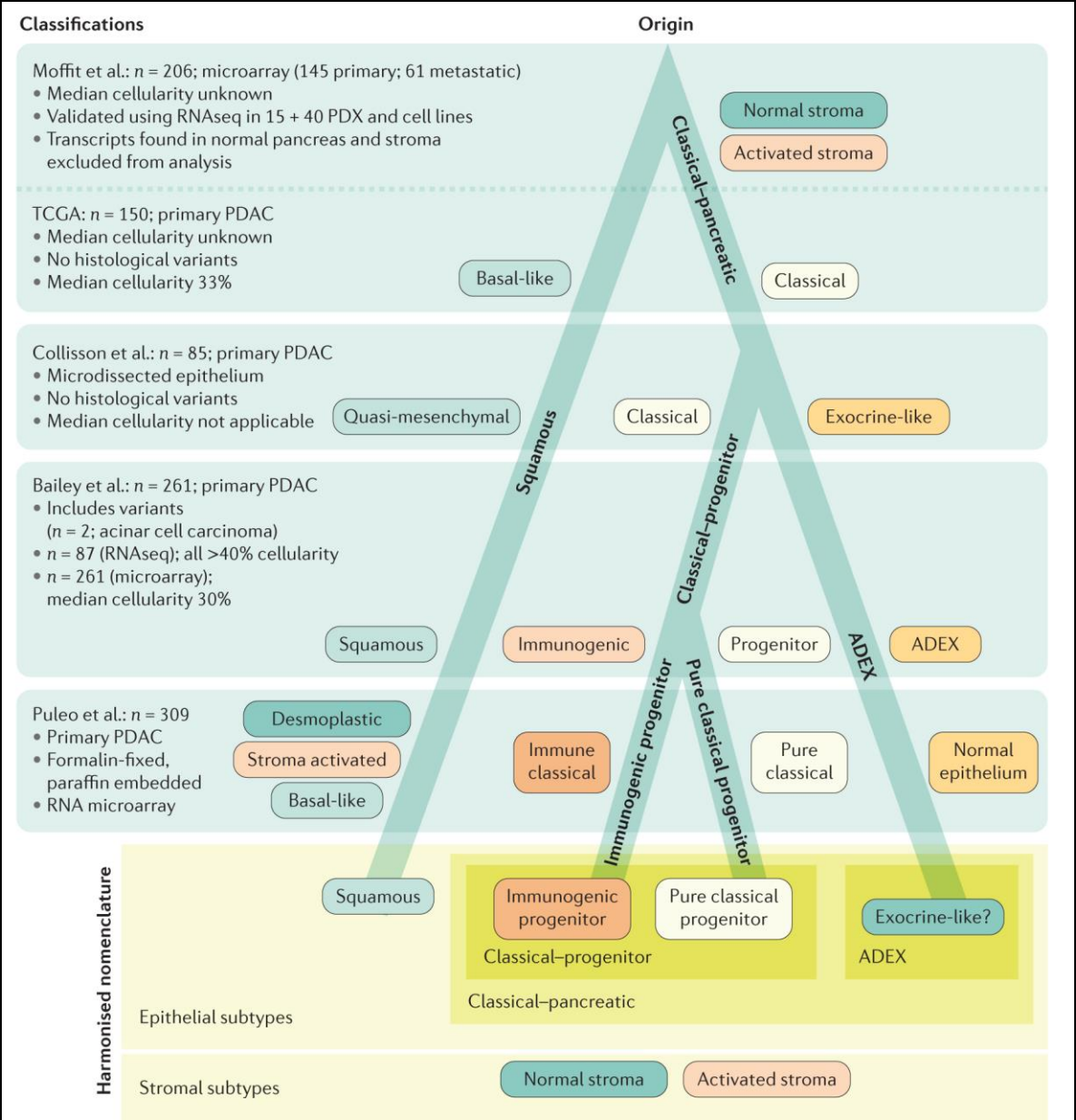
Acinar Cell
Carcinoma

IPMN associated PDAC
Colloid PDAC

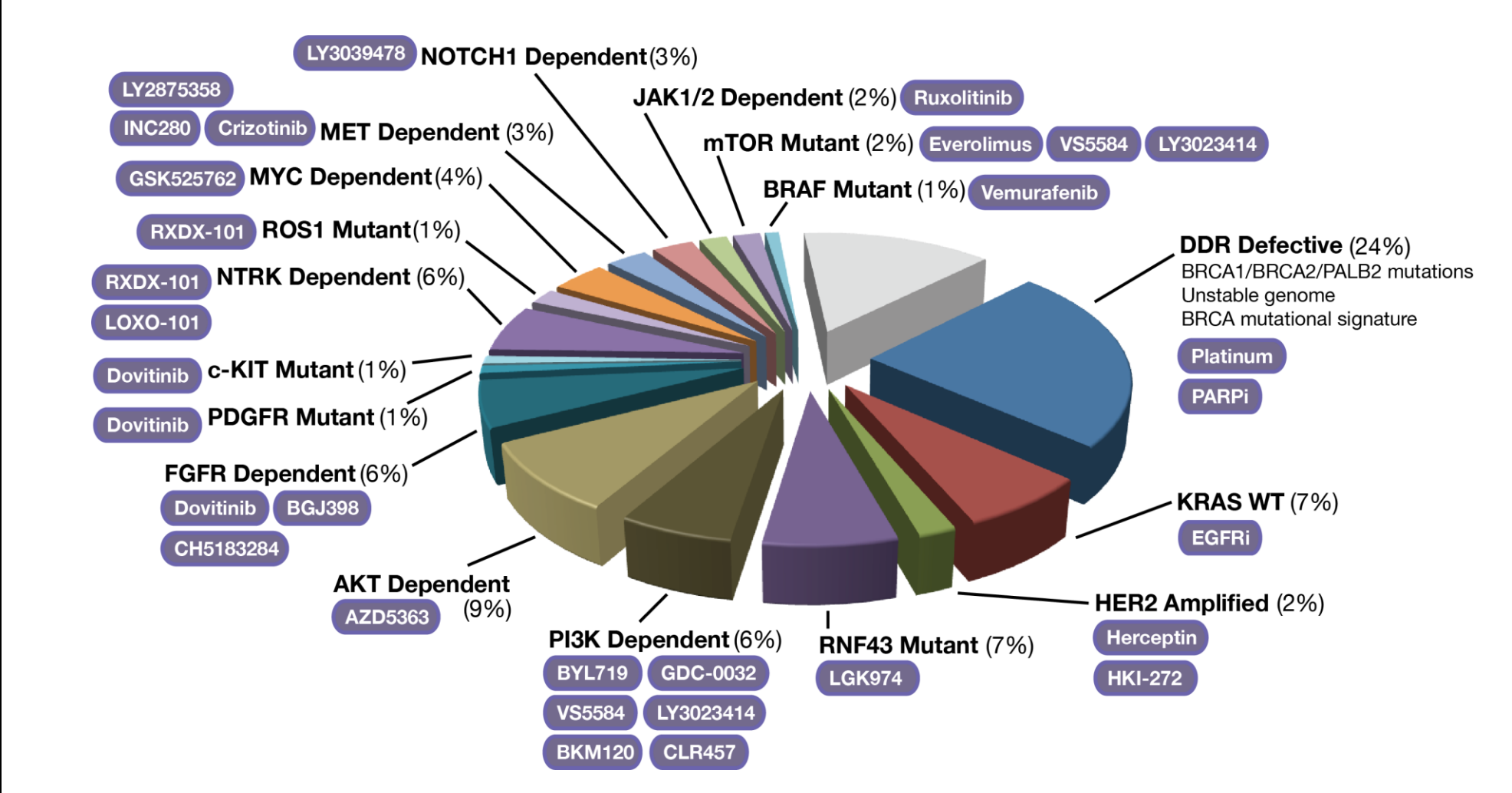
Pancreatic Cancer Phylotranscriptomic Tree



Molecular subtypes of pancreatic cancer.
Collisson EA, Bailey P, Chang DK, Biankin AV.
Nat Rev Gastroenterol Hepatol. 2019;16:207-220.



Pancreatic Cancer “Druggable Genome” (primary resectable)

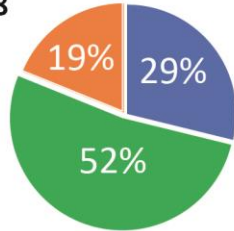


High S100A2/A4 = poor response to surgery

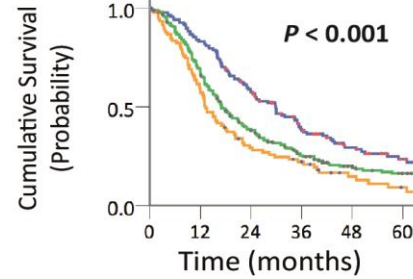
Biomarker Validation ($n = 1184$)

A.

APGI Cohort
 $n = 518$



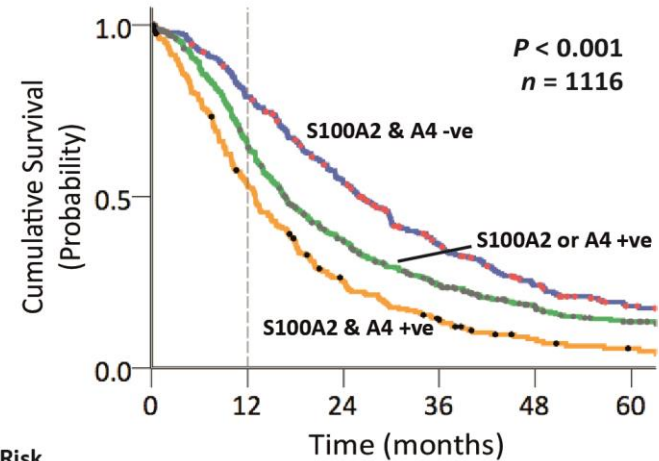
B.



C.

■ S100A2 & A4 -ve
■ S100A2 or A4 +ve
■ S100A2 & A4 +ve

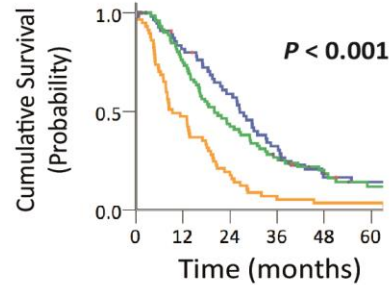
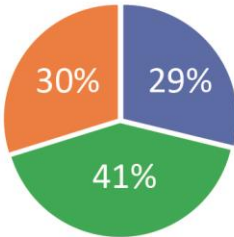
Survival at 12 months 79% vs 66% vs 54%



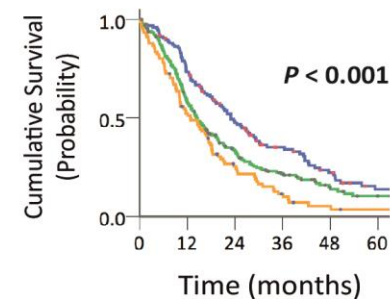
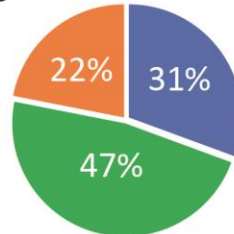
No at Risk

S100A2 & A4 -ve	329	248	162	93	54	30
S100A2 or A4 +ve	542	339	181	103	56	31
S100A2 & A4 +ve	245	127	56	29	13	7

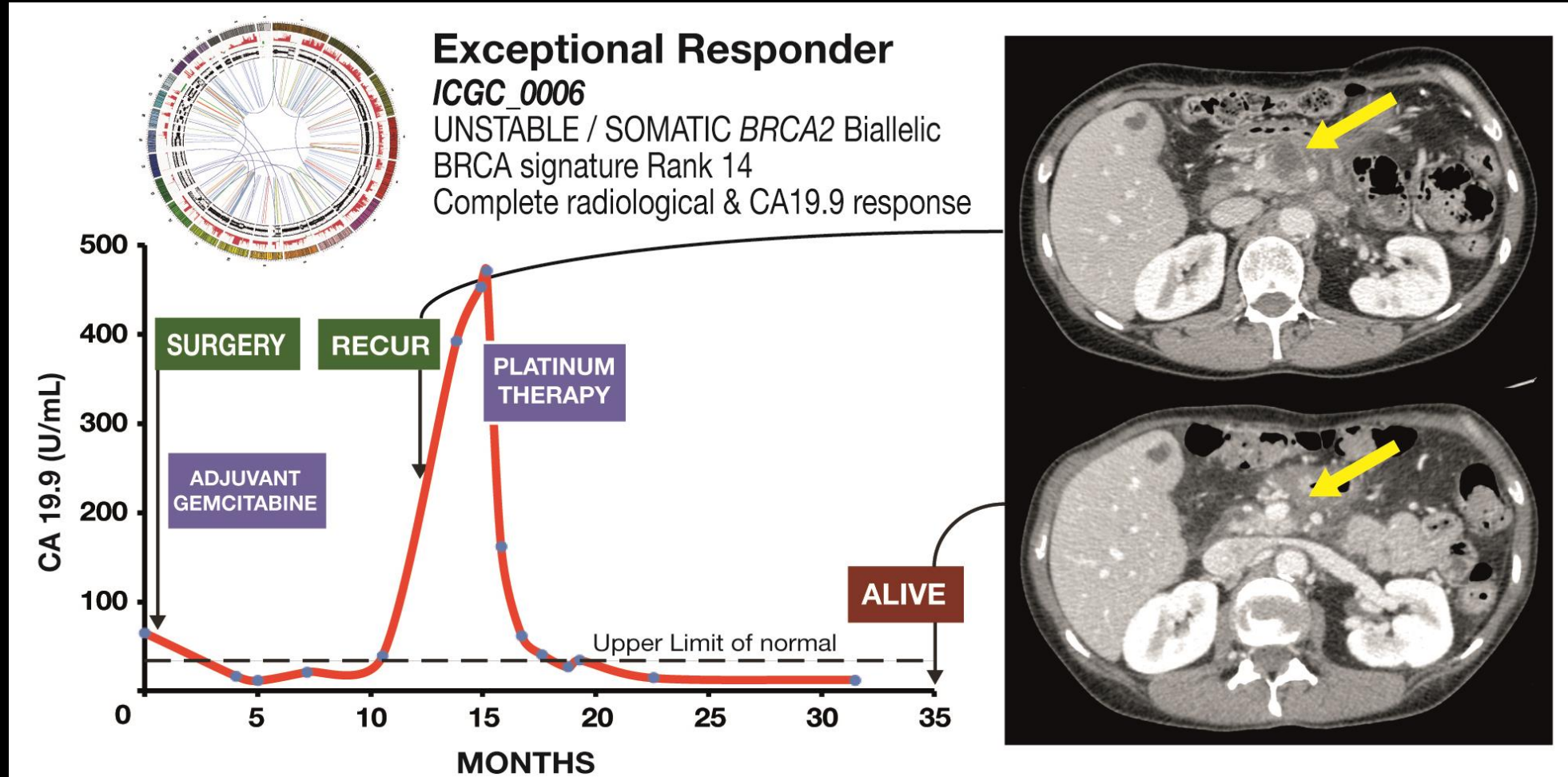
Glasgow Cohort
 $n = 198$



German Cohort
 $n = 468$



Pancreatic Cancer (64 year old female)



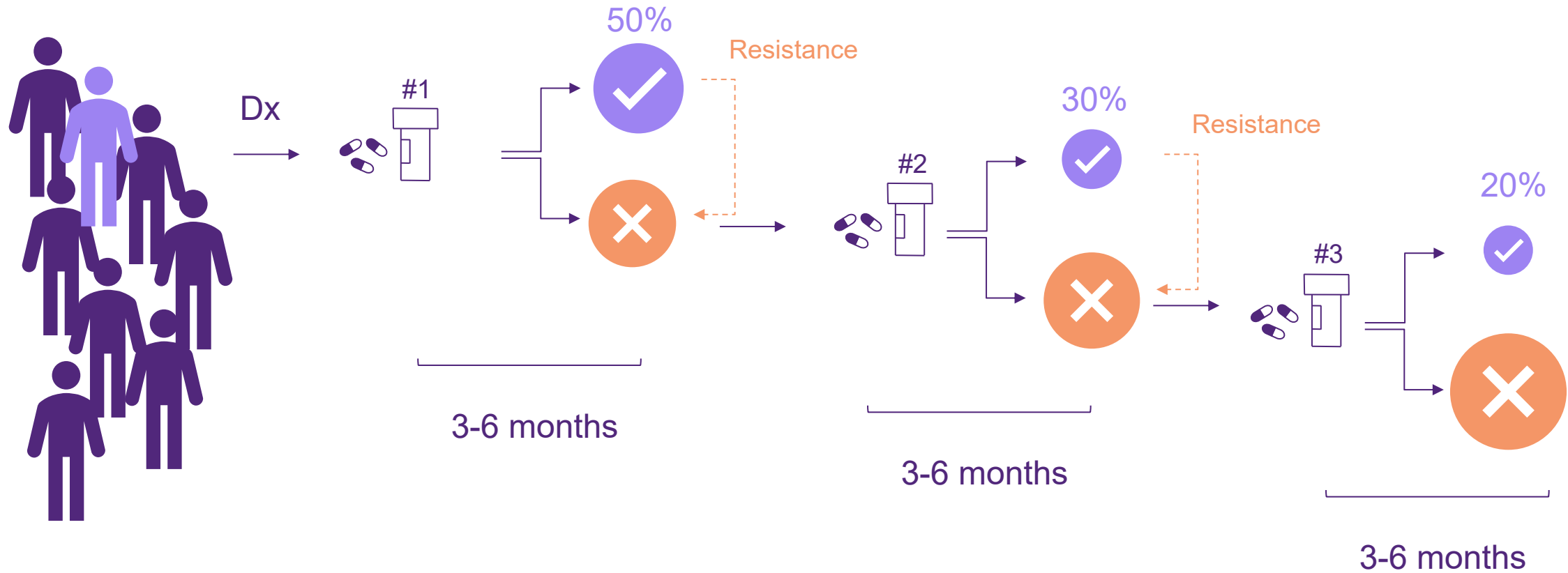
Sustainability

Better outcomes

Quality of life

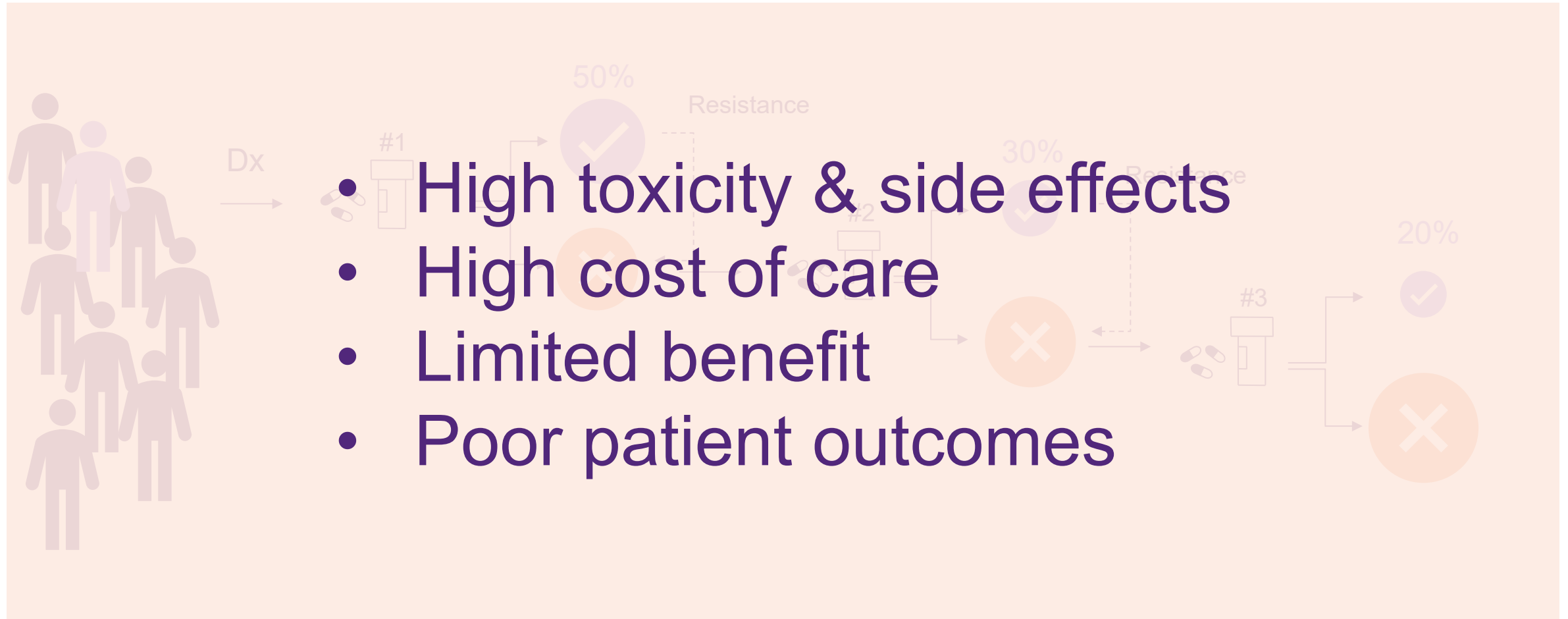
Trial and error based on **average** response rates

Cannot predict **ahead of time** if a patient is going to respond to a treatment or have best possible outcome



Trial and error based on **average** response rates

Cannot predict **ahead of time** if a patient is going to respond to a treatment or have best possible outcome



Why individual patient response is important

Treatment Options	Overall Response Rates	Individual Patient Response Rates				
		Mrs A.S.	Mr J.W.	Ms P.B.	Mr F.A.B.	Prof A.V.B
Treatment A	55%	5%	95%	22%	9%	5%
Treatment B	50%	18%	11%	77%	9%	2%
Treatment C	42%	85%	2%	16%	6%	1%
Treatment D	32%	22%	29%	9%	2%	6%
Clinical Trial	?	45%	30%	17%	63%	8%
						?

Increasing value of better decisions

Current oncology drug development process is expensive, long and has high attrition rates

Phase	Failure Rate	% Dev Cost	Cost (\$, m)	Cost of failure (\$, m)	Attrition (\$, m)
Preclinical	60%	7%	\$140	\$11	\$84
Phase 1	35%	15%	\$300	\$59	\$105
Phase 2	50%	21%	\$420	\$126	\$210
Phase 3	40%	26%	\$520	\$312	\$208
Overall	99.5%	86%	\$2,000	-	\$927

Source: Adapted from Sun et al (2022), Wouters et al (2020)

DATA, DATA, DATA ...

**Real World Data
Prospective Data
Clinical Trial Data
Molecular Data**

The next generation of cancer tests

The Glasgow Cancer Test



Susie Cooke
Deputy Director
Medical Genomics
and Informatics



Andrew Biankin
Director
Therapeutic and Diagnostic
Development



Philip Beer
Clinical Cancer Genomics

Clinical Trial Platforms



CANCER
RESEARCH
UK

PRECISION
PANC

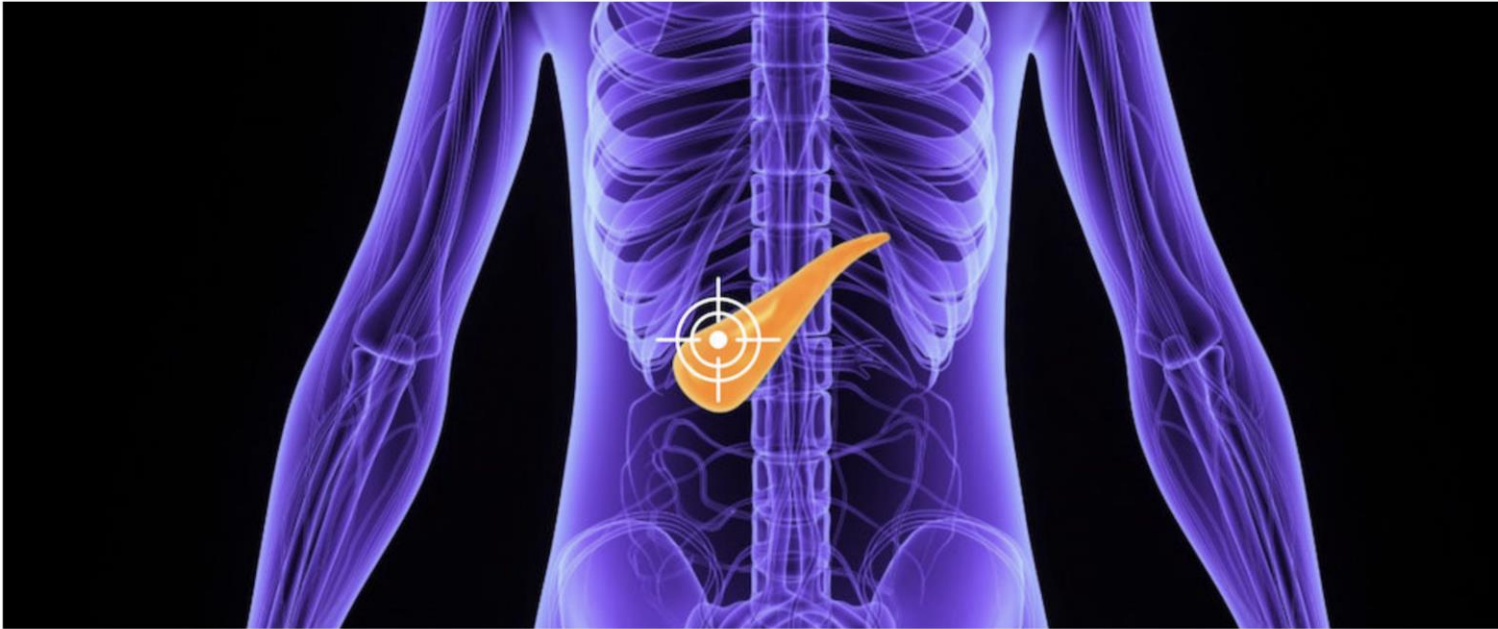


About us

Patients & Carers

Clinicians Area

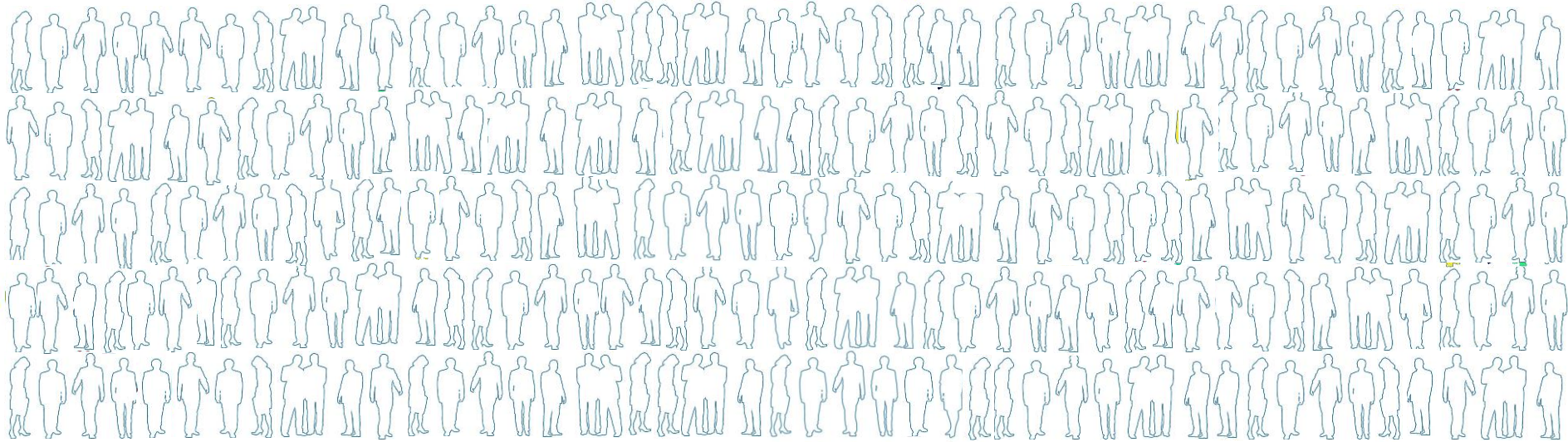
News & Events



Precision-Panc

Not all cancers are the same. Precision medicine is about tailoring treatments to an individual's cancer. Precision-Panc Clinical Trials are delivered through the NHS and match people with pancreatic cancer to the trial most likely to work for them.

From single to multiple gene tests



TEST FOR MANY GENES AT ONCE

Novel
Targets
and
Drugs

Trial of
Drug A

Trial of
Drug B

Trial of
Drug C

Trial of
Drug D

Harmonising world-wide efforts to reduce the global burden of cancer

The ARGO project is a new phase of the International Cancer Genome Consortium; translating genomic knowledge into new approaches to improve outcomes for people affected by cancer.

[FIND OUT MORE](#)

Accelerating Research in Genomic Oncology

ICGC ARGO will uniformly analyze specimens from 100,000 cancer patients with high quality clinical data to address outstanding questions that are vital to our quest to defeat cancer.

26 Programs, 13 countries, 63,116 donors committed



More details on member programs:

<https://www.icgc-argo.org/page/89/project-list>

COMMENT | [VOLUME 24, ISSUE 2, P123-125, FEBRUARY 2023](#)

Accelerating cancer omics and precision oncology in health care and research: a *Lancet Oncology* Commission

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Published: February, 2023 • DOI: [https://doi.org/10.1016/S1470-2045\(23\)00007-4](https://doi.org/10.1016/S1470-2045(23)00007-4) •



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International Commission to Expand Molecular Testing Access for Cancer Patients

Jan 31, 2023 | [staff reporter](#)

NEW YORK – The University of Glasgow said on Tuesday that it will lead a new commission dedicated to improving global access to diagnostic tests and personalized cancer care.

The Lancet Oncology Commission, comprising an international team of experts, will study obstacles to widespread adoption of molecular testing for cancer diagnosis and treatment and generate practical solutions for expanding access and reducing global health inequities. The commission will analyze how health and genomic data are collected and shared in order to develop best practices and facilitate drug development.

University of Glasgow to lead international effort to advance cancer care and research

Hippocratic Post | 31st January 2023 | CANCER, GLOBAL REACH | No Comments

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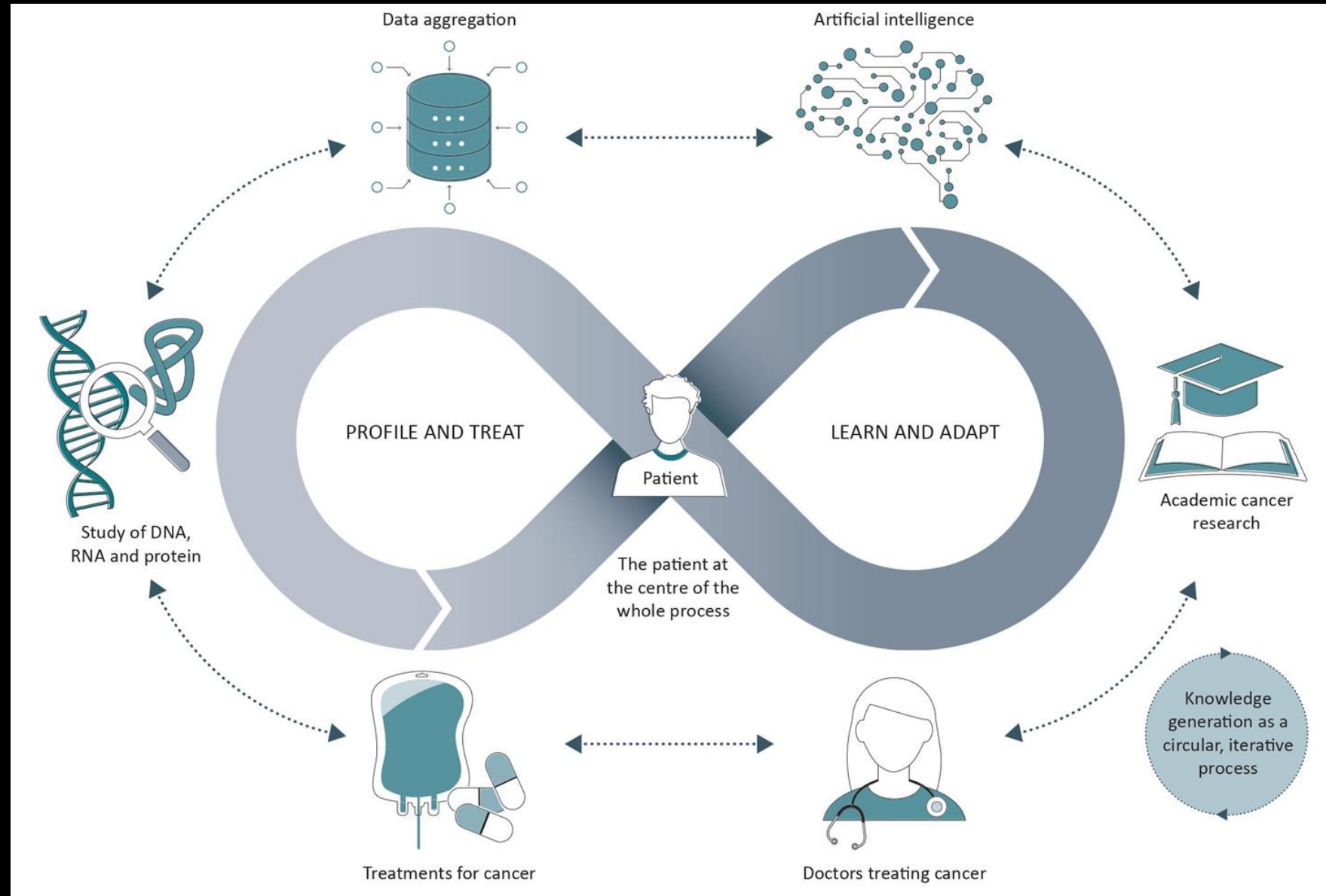
International Commission to Expand Molecular Testing Access for Cancer Patients

Jan 31, 2023 | [staff reporter](#)

NEW YORK – The University of Glasgow said on Tuesday that it will lead a new commission dedicated to improving global access to diagnostic tests and personalized cancer care.

Bridging the implementation gap

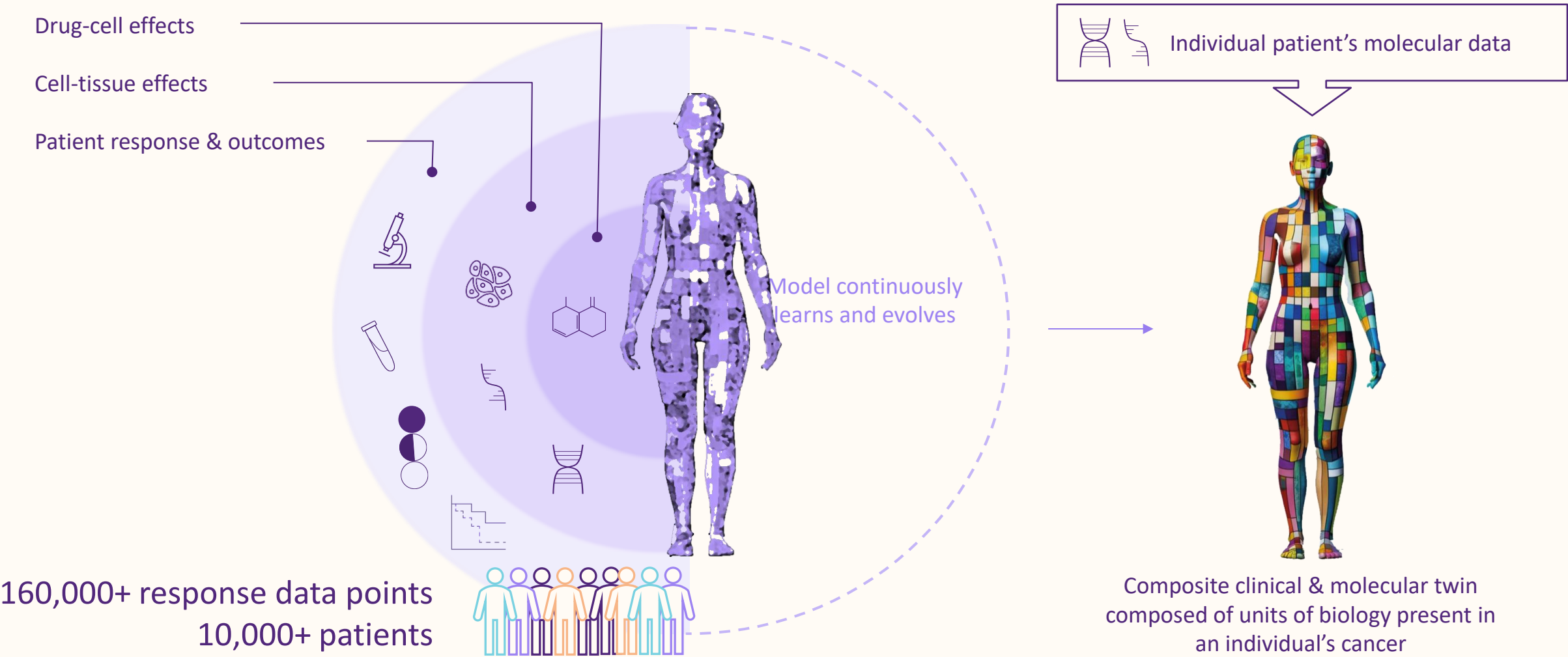
Taken from:
Beer and Biankin, Bridging the implementation gap: delivering complex genomic analysis for routine cancer care. *Annals of Oncology*, *in press*.



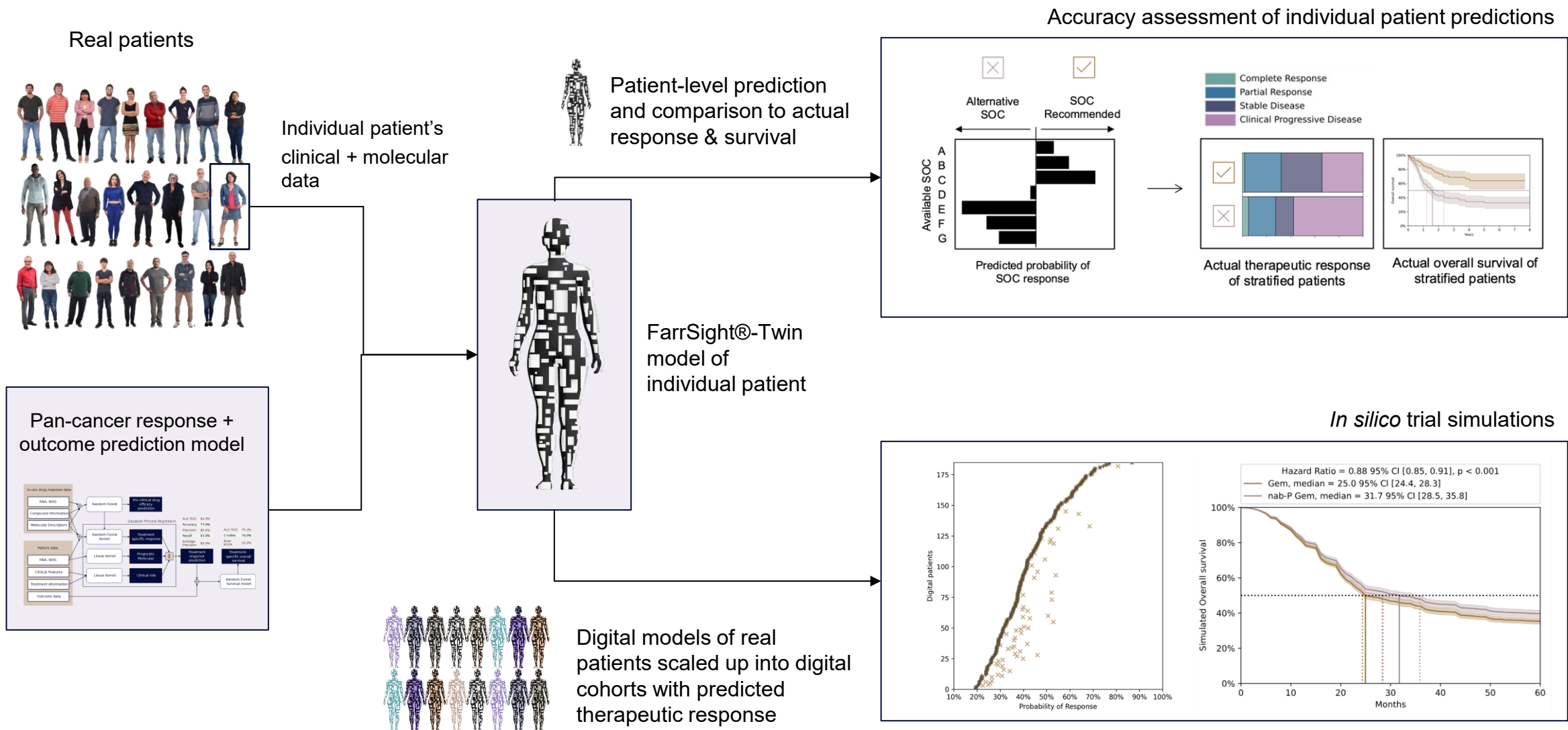
AI in Healthcare and Therapeutic Development

Digital Twins of Cancer Patients

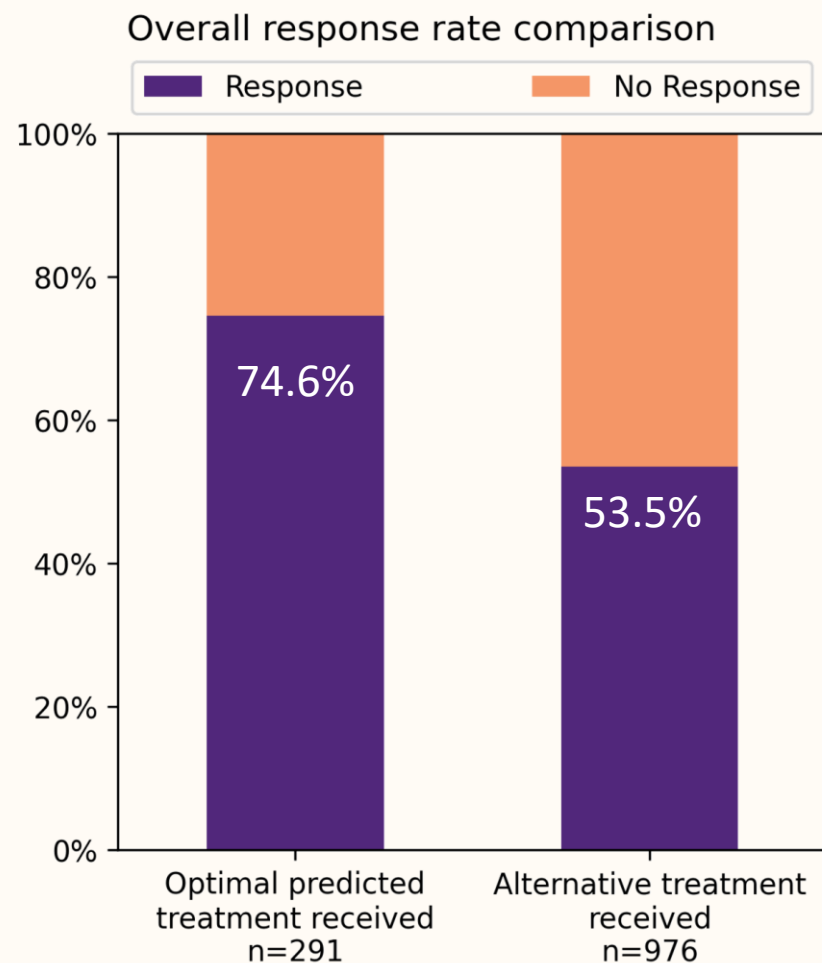
FarrSight®-Twin: the next-generation of digital twin to predict individualised therapeutic response and outcome



FarrSight®-Twins of each and every individual patient



FarrSight®-Twin-predicted treatment = better overall response

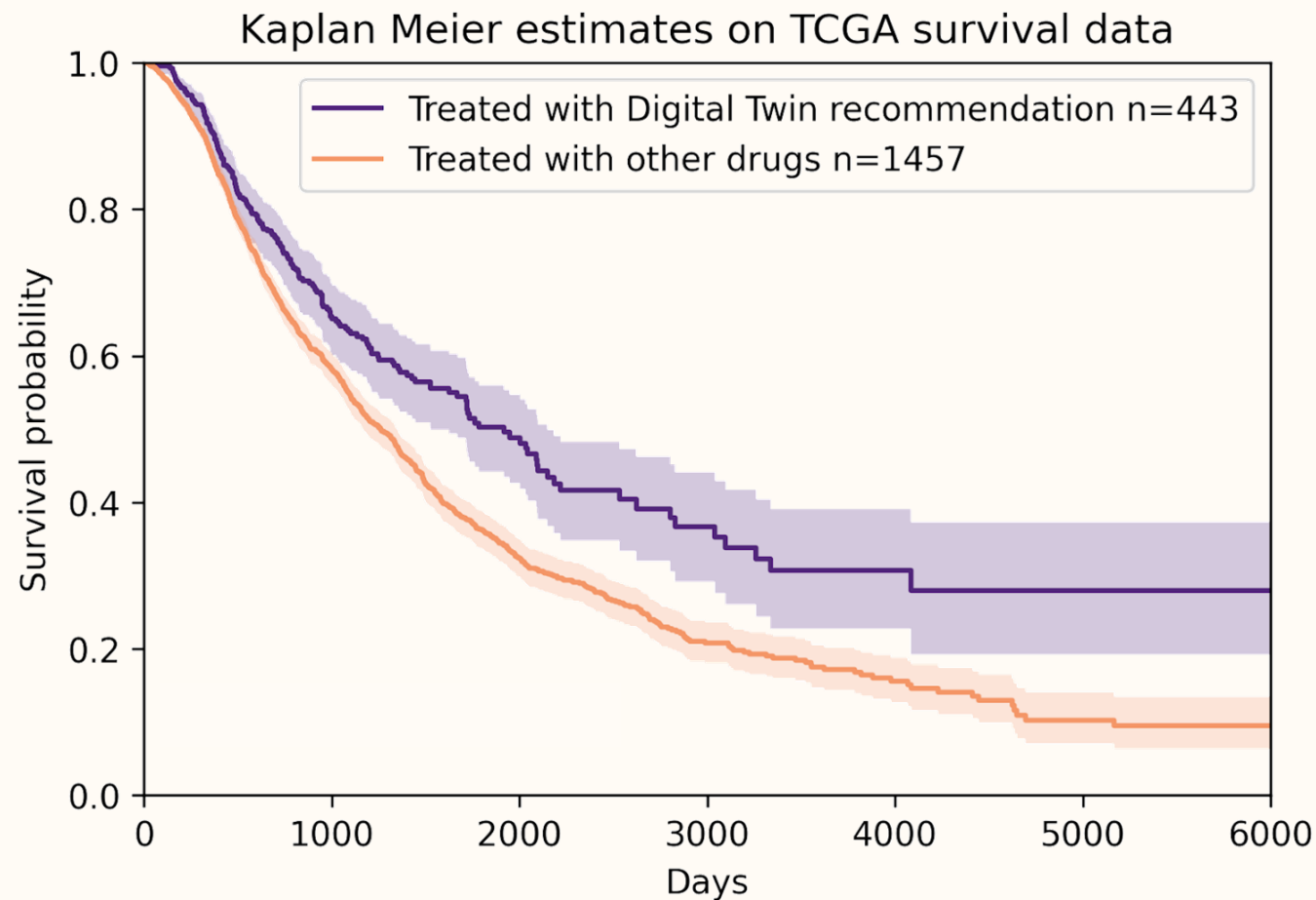


Odds ratio = 2.55

Fisher exact $p=1 \times 10^{-10}$

Significantly higher overall response rates in patients who received treatment recommended by FarrSight®-Twin compared to patients who didn't receive the treatment predicted to be optimal

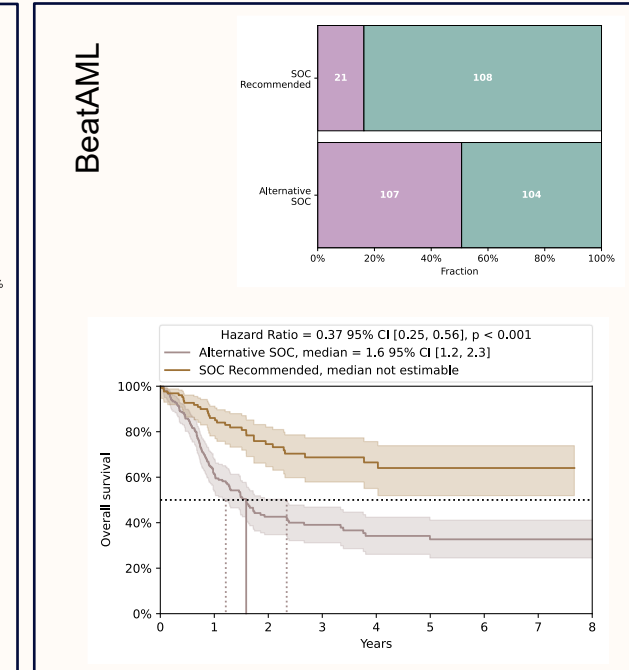
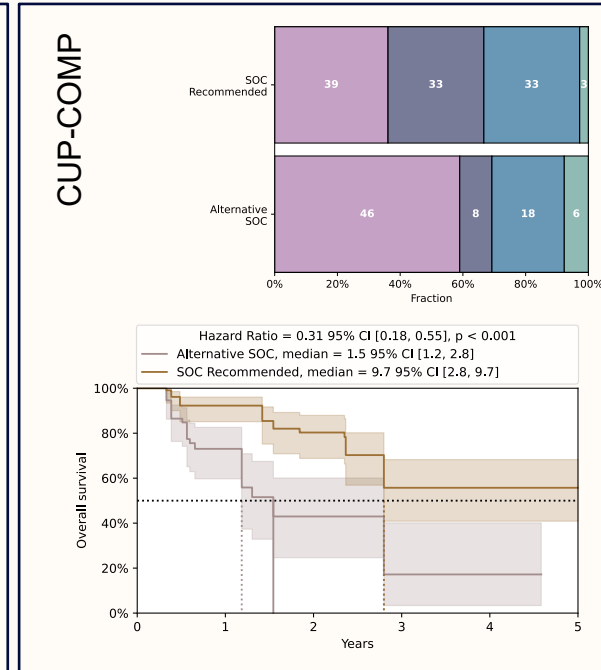
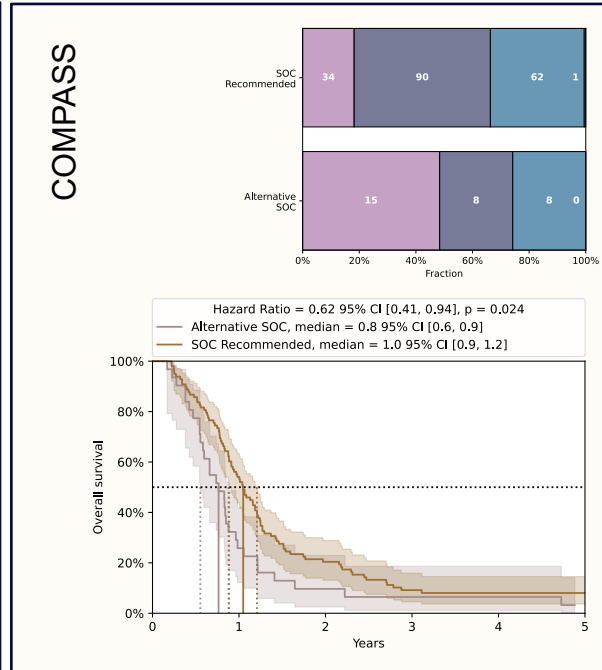
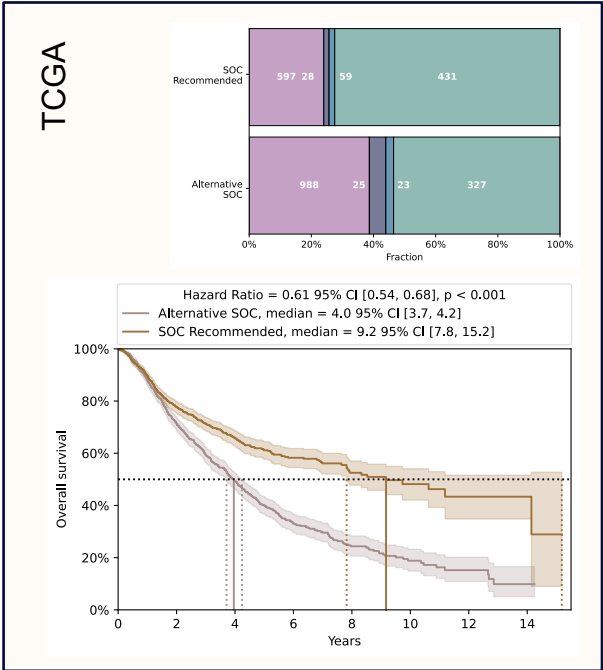
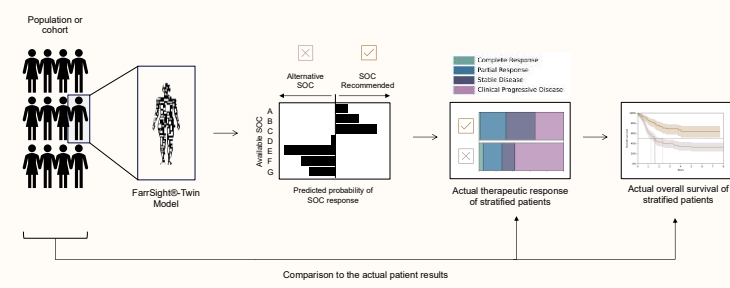
Optimal FarrSight®-Twin-predicted treatment = better survival



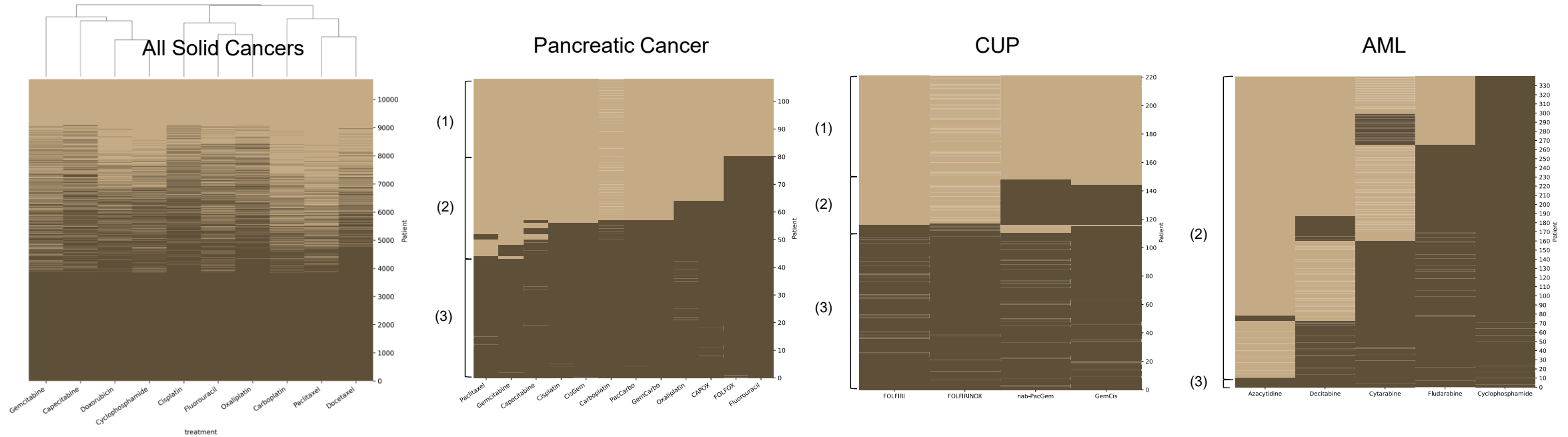
Log-rank p-value=2.5E-6

Significantly better overall survival in patients who received FarrSight®-Twin-predicted therapy compared to those who did not

FarrSight®-Twin predicts optimal therapy across datasets

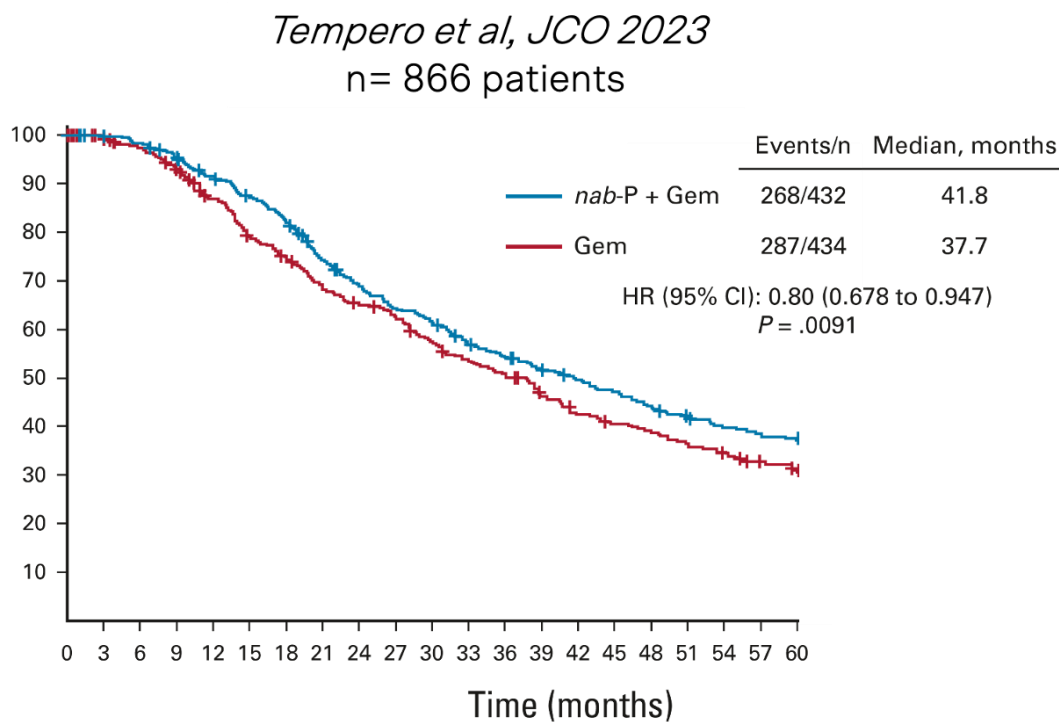
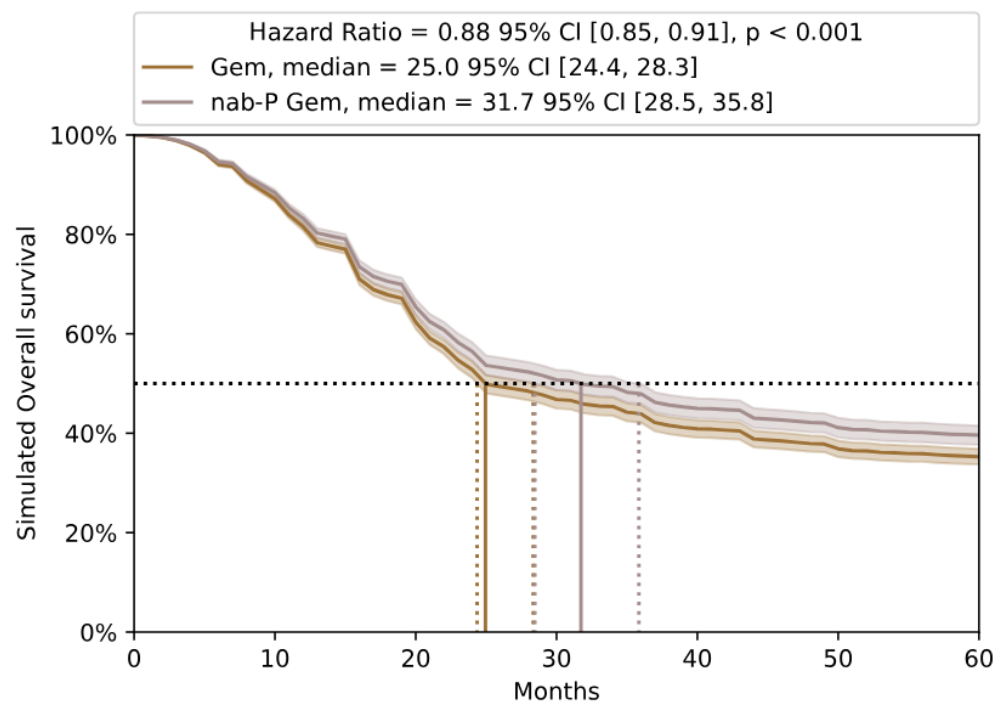


Chemotherapy Response (all chemotherapies)

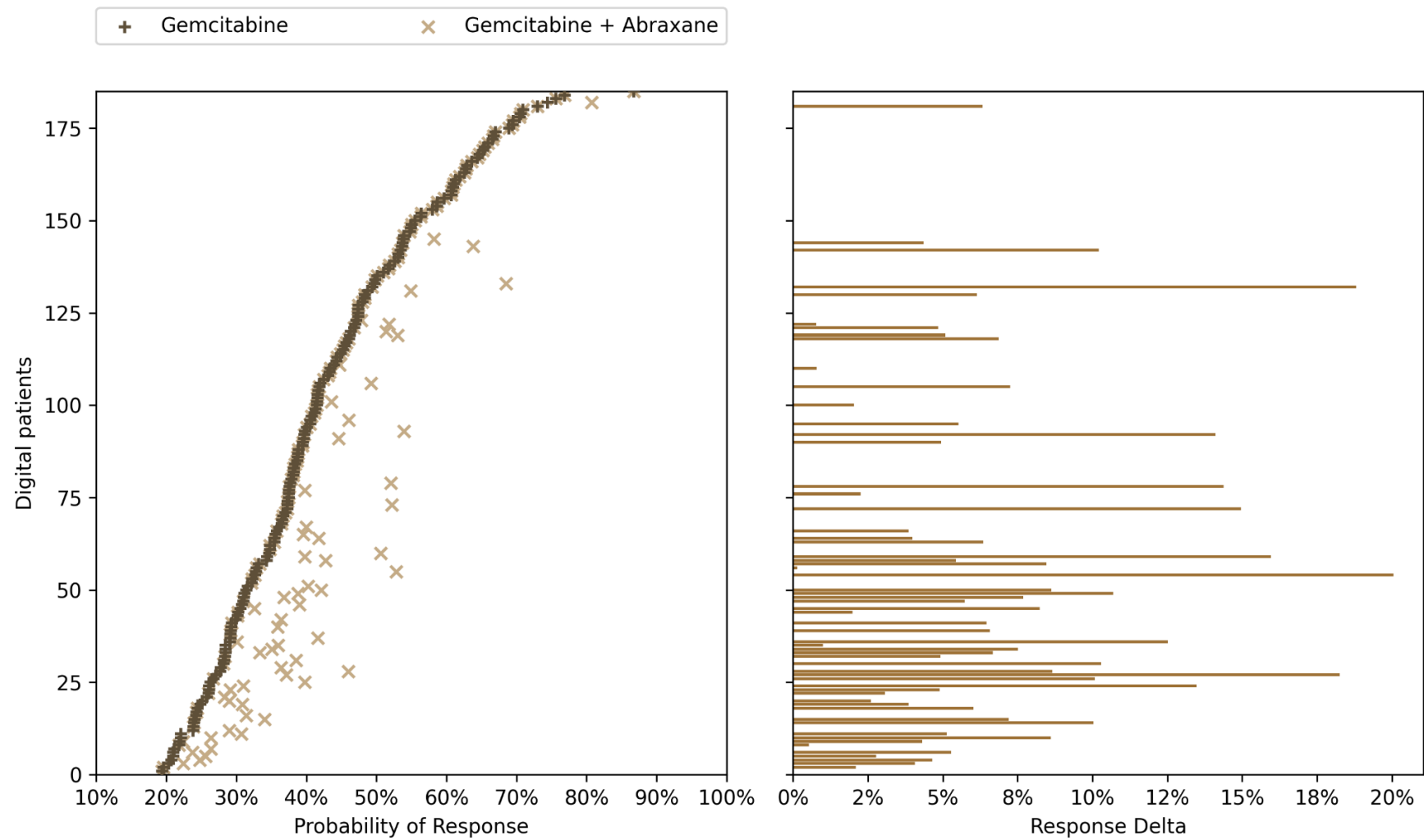


Predicted responses across multiple cancer types

Application in research clinical trials: RWD for trial simulation



FarrSight®-Twin scaling to cohorts captures real-world response heterogeneity



Transforming Healthcare:

Use current treatments better
Predicting clinical trial and “off label” options

Select drugs for development
Select indications
Understand MOA and Biology
Biomarker discovery and development
Synthetic controls
Single arm combination studies
Clinical trial design

Transforming Healthcare (Challenges):

Adoption:

**What evidence is required?
Compared to What?**

Regulators and Payers

Integration with current systems

Inertia



University
of Glasgow

Thank you

